

U.S. Department of State / GHSD — Advancing Global Health (DFOP0017890)

Statement of Interest — Phase 1

FairBanks Medical Centre · FairBanks Community Reach · FCHIP

Track 2, Objective 4: Strengthen and Scale Interoperable, Fit-for-Purpose Digital Health Systems

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Table Listing of Critical Details

Item	Detail
Proposed Project Title	FCHIP Last-Mile Bridge: Interoperable Community Health Data into Uganda's National Digital Health Architecture
Name of the Organization	FairBanks Medical Centre
Addendum to which the SOI is responding	Addendum E: Health Foreign Assistance Memorandum of Understanding (MOU) Implementation in Uganda
Target Benefiting Country	Uganda
Total Federal Share Requested	\$1,850,000
Total Cost Share (as applicable)	\$0 (none proposed at SOI stage)
Project Length	36 months

Opportunity: DFOP0017890 | Window 2a SOI due 31 July 2026, 5:00 PM EDT | This cover table does not count toward the 5-page narrative limit.

1. Issue / Challenge / Opportunity

Uganda's five-year health transition (April 2026-December 2030) asks facilities, districts, and communities to keep HIV, TB, malaria, maternal and child health, immunisation, and outbreak services strong while shifting toward government-led, performance-based programming. That shift only works if decision-makers see what is happening at the last mile - not weeks later in paper registers.

Community Health Workers and Village Health Teams already walk households and refer patients. Too often their work never reaches eCHIS, facility EMRs, DHIS2/eIDSR, or the National Data Warehouse in a usable form. National digital systems then miss early fever clusters, missed antenatal visits, immunisation gaps, and stock pressure. The result is reactive care, weak accountability, and higher risk that transition gains reverse.

The opportunity is clear: bridge last-mile community data into Uganda's approved National Digital Health Architecture (NDHA) so Ministry of Health (MoH) systems become more complete, more actionable, and progressively owned by Government of Uganda (GOU) institutions - without building a parallel silo.

2. Proposed Solution / Activities

FairBanks Medical Centre and FairBanks Community Reach respectfully submit this Statement of Interest under Track 2, Objective 4 of Addendum E: strengthen and scale interoperable, fit-for-purpose digital health systems with progressive GOU responsibility and sustainable operations.

We propose to pilot and then transfer a community-to-facility intelligence layer - the FairBanks Community Health Intelligence Platform (FCHIP) - that captures CHW/VHT household and visit data offline, syncs securely, maps indicators to national reporting elements, and feeds facility and district action. FCHIP is not a parallel national system. It is a bridge that makes Uganda's existing digital stack more complete at community level and more useful for prevention.

Primary subsystem focus (permitted under Objective 4): eCHIS-aligned household/visit capture; facility EMR encounter and referral handoff (eAfya/ClinicMaster pathways as available in the catchment); DHIS2 aggregate reporting and eIDSR surveillance signals; readiness feeds toward the National Data Warehouse; and iHRIS-aware CHW/VHT workforce directories where schemas allow. Design aligns with NDHA and OpenHIE-consistent interoperability (API-first, standards-aware exports), under MoH digital health authority.

This partnership is designed as a durable win-win: each party gains something lasting, and no party depends forever on a private tool outside GOU systems.

Who gains	What they gain
Uganda / Ministry of Health	Richer last-mile data into eCHIS, DHIS2/eIDSR, and related national platforms; clearer referral closure; documented handoff of staffing and recurrent costs toward GOU ownership by project close.

Who gains	What they gain
U.S. Department of State / GHSD	A Ugandan implementer with a live clinic and community network that protects essential services while proving interoperable digital scale-up under the MOU - accountable, measurable, and transferable - advancing U.S. interests in health security and country self-reliance.
Districts & facilities	Dashboards for outreach targeting, referral follow-up, and programme monitoring; GIS hotspot views that support planning before outbreaks arrive at the ward.
Communities & CHWs/VHTs	Simple offline tools, faster support from the medical centre, and care that reaches mothers, children, older persons, and people with chronic conditions before crises.
FairBanks / FCHIP	Room to refine and document a production-grade interoperability model inside a real catchment - then share specifications, training packages, and transfer plans with MoH-aligned partners.

Core activities:

- A1. Deploy offline-capable CHW/VHT capture aligned with eCHIS household and visit concepts across FairBanks Community Reach catchments.
- A2. Establish a secure sync and role-based access pipeline linking community records to FairBanks Medical Centre workflows and authorised district viewers, consistent with Uganda data-protection and cybersecurity expectations.
- A3. Map community indicators to DHIS2 reporting elements and demonstrate aggregate submission pathways for MNCH, immunisation, NCD screening, and disease surveillance signals (including eIDSR-relevant alerts).
- A4. Deliver facility and programme dashboards plus GIS hotspot views that trigger outreach, referrals, and supply planning actions.
- A5. Produce interoperability specifications, data-governance protocols, training curricula, and a staged GOU-ownership transition plan (staffing and recurrent operating costs) suitable for full-proposal detailed design.

Project approach:

Anchor in a live cascade: community members identify needs; CHWs/VHTs collect and educate; FairBanks Community Reach runs outreach and home visits; FairBanks Medical Centre provides clinical care, diagnostics, pharmacy, and referrals; FCHIP is the intelligence layer that closes the data-and-feedback loop.

Build API-first, standards-aware exports - not a closed proprietary vault. Prioritise interoperability with eCHIS, DHIS2/eIDSR, facility EMR pathways (eAfya/ClinicMaster as applicable), and National Data Warehouse readiness when schemas are confirmed.

Protect essential services during transition: structure community data around HIV/TB/malaria flags where relevant, maternal and child pathways, immunisation follow-up, and outbreak early warning so digital work reinforces - not distracts from - service continuity.

Co-design forms and alerts with CHWs, facility staff, and local leaders; train Ugandan data stewards and developers; document every interface for MoH review.

Start where FairBanks already operates (Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya and surrounding communities), then design for district replication using Uganda's existing CHW/VHT architecture.

3. Anticipated Outcomes and Results

Success means lives protected, systems strengthened, efficiency improved, self-reliance advanced, and U.S. investments secured through GOU-owned digital continuity:

Outcome lens	What success looks like
Save lives	Earlier alerts for missed ANC, immunisation gaps, fever clusters, and referral drop-offs - linked to CHW follow-up and clinic action.
Strengthen systems	Working mappings and demo exports for eCHIS-aligned records and DHIS2/eIDSR aggregates, with open documentation for MoH and partners.
Enhance efficiency	Less double entry and paper lag; role-based dashboards that shorten the path from community signal to facility or district response.
Foster self-reliance	Handoff package: source documentation, SOPs, training, hosting options, and a costed path to transfer recurrent operations toward domestic systems by project close.
Advance U.S. interest	Measurable contribution to MOU digital transition: interoperable last-mile data under GOU authority, protecting prior U.S. health investments and strengthening epidemic early warning.

4. Organizational Capacity

- Ugandan social enterprise with a functioning medical centre - clinical care, diagnostics, pharmacy, and referral pathways already in daily use.
- Active FairBanks Community Reach programmes: maternal and child health, Gericare, chronic disease screening, school and corporate health outreach.
- Established CHW/VHT relationships in Kampala-area communities named above - real users, not a greenfield pilot.
- Digital health records foundation and pharmacy dispensing data already supporting facility operations.
- Research orientation and partnership practice - evidence, ethics, and skills development sit inside the FairBanks community health model.

5. Interoperability Pathway (chart)

Platform	Integration pathway
eCHIS	Household registration, visit logs, referral flags via structured mobile capture
Facility EMR (eAfya / ClinicMaster)	Referral handoff and encounter linkage at FairBanks Medical Centre

Platform	Integration pathway
DHIS2 / eIDSR	Aggregate exports and surveillance signals for MNCH, immunisation, outbreaks
National Data Warehouse	Standardised JSON/API feeds when national schema is confirmed
iHRIS	CHW/VHT workforce directory alignment where schemas allow



Figure 1. Community cascade to FCHIP to national digital health systems (chart; excluded from page limit).

6. Alignment with Addendum E Guiding Principles (chart)

Principle	How FairBanks responds
Sustainability & country ownership	Design for MoH/NDHA alignment and staged transfer of operations
Protect essential services	Digital workflows reinforce HIV, TB, malaria, MNCH, immunisation
Data for action & accountability	Quality data into GOU systems - used for decisions, not parallel burden
Integrated, people-centred care	One community-to-facility loop; less fragmentation
Local partnerships	Government, CHWs/VHTs, CBOs, academic and NGO partners
Evidence-based & context fit	Pilot in live catchments; measure; refine before scale
Oversight & coordination	Align with MoH priorities and designated review platforms

7. List of Partner Roles and Responsibilities (preliminary)

Partners below are preliminary characterisations for Phase 1. Roles, contacts, and any subrecipient federal-share splits will be refined in Phase 2 / co-design if invited. At SOI stage, FairBanks Medical Centre is proposed as prime applicant.

Partner	Proposed role (summary)
Ministry of Health (digital health / community health)	Alignment with NDHA, eCHIS, and national reporting; review of interfaces and transfer plan
District health teams (pilot geography)	Aggregate reporting validation, dashboard use, readiness dialogue

Partner	Proposed role (summary)
CHWs, VHTs, and community leaders	Primary data partners; co-design of forms and alerts
Academic partners (to be confirmed at Phase 2)	Evaluation support and interoperability testing
NGOs / CBOs collaborating with FairBanks Community Reach	Outreach coordination and community engagement

8. Resource Contributions and/or Cost Share

No cost share is proposed at the SOI stage. FairBanks will continue operating its medical centre and community outreach as complementary organisational activity. Opportunities for in-kind or matching contributions can be discussed during co-design or Phase 2 if invited.

9. Readiness for Phase 2

This Statement of Interest confirms FairBanks' intent to advance Track 2, Objective 4 with a community-rooted, interoperable, government-aligned digital health proposal. We seek invitation to Phase 2 (full proposal) and/or consultative program design, where we will detail workplans, budgets, M&E, partnership letters, and a clear transition of staffing and recurrent costs toward sustainable Ugandan ownership.

References: DFOP0017890 APS (Phase 1 SOI format) and Addendum E (Uganda). Suggested upload name: FairBanks - SOI - Addendum E.pdf

<https://opportunitiesforyouth.org/2026/06/15/u-s-department-of-state-launches-up-to-60-million-funding-opportunity-to-strengthen-ugandas-health-system-through-the-health-foreign-assistance-mou-implementation-plan/>
<https://simpler.grants.gov/opportunity/44a573b8-d46a-4cec-8c84-f319cd1cc1b6>

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